



ORIGINAL ARTICLE

Time Together as an arena for mental health nursing – staff experiences of introducing and participating in a nursing intervention in psychiatric inpatient care

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ABSTRACT: A lack of meaningful activities for people with mental ill health admitted to psychiatric inpatient care has been related to feelings of boredom and ‘doing nothing’ and is not in line with recovery-oriented care. Staff in psychiatric inpatient care report having limited time, ambiguous responsibilities, and insufficient support that counteracts their ideals of good nursing care and puts them at risk for high levels of stress and stress of conscience. Research highlights a need for interactions between patients and staff, but few nursing interventions with such a focus are described in the literature. This qualitative study aimed to illuminate staff experiences of introducing and participating in the nursing intervention Time Together, via qualitative content analysis of 17 individual semi-structured interviews with nursing staff in psychiatric inpatient care. The results show that these staff members experienced Time Together as an arena for mental health nursing. They prepared for the introduction of the intervention by laying a framework for success. Although the actual implementation led to them feeling burdened, they found that Time Together fostered relationships between patients and staff. For successful implementation, mental health nurses need to advocate the intervention. As Time Together constitutes an arena for mental health nursing, play and conversations based on reciprocity and equality can contribute to patients’ recovery.

KEY WORDS: engagement, mental health nursing, nursing intervention, psychiatric inpatient care, Time Together.

BACKGROUND

A lack of meaningful activities for people affected by mental ill health admitted to psychiatric inpatient care (PIC) is related to waiting and boredom (Folke *et al.* 2017; Lindgren *et al.* 2015), not in line with recovery-oriented care. According to patients in PIC, inactivity counteracts their recovery but PIC staff lack time for interactions with patients and seem to have competing medical and administrative commitments (Molin *et al.* 2016a). PIC staff themselves agree, expressing that limited time, ambiguous responsibilities, and insufficient support counteract their ideals of care (Cleary, 2004;

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Graneheim *et al.* 2014). There is a risk that staff will become resigned and focused on their own survival when hindered from working in line with their ideals (Molin *et al.* 2016b). Similarly, other authors have concluded that PIC nurses might be demoralized if their workplace becomes too challenging, with little time to see the patients (Cleary, 2004; Cleary *et al.* 2012; Gabriellson *et al.* 2016). However, stress among staff might decrease if they are able to devote time to things they consider important; that is, being with the patients (Tuveesson *et al.* 2011).

Researchers have emphasized that PIC staff need to spend more time with patients in joint activities, for the benefit of both patients and staff. There is an imbalance in the amount of PIC nurses' time taken up by activities other than being with the patients (McAndrew *et al.* 2014). Gunasekara *et al.* (2014) stressed a need for renewed attention to the basics of relationships between patients and staff in PIC and urged staff to prioritize time to get to know the patients. Staff engagement could provide a basis for safe and recovery-oriented care for PIC patients (Polacek *et al.* 2015).

One response to this situation was a project which introduced and evaluated the nursing intervention Time Together (TT) (Molin *et al.* 2017), which is based on the structures of Protected Engagement Time (PET) (Nolan *et al.* 2016) and Barker and Buchanan Barker's theory of engagement (2005). The predetermined structure of TT is presented in Figure 1.

This study comprises a qualitative evaluation of the introduction and participation in TT in Swedish PIC. Other parts of the evaluation are presented elsewhere. Molin *et al.* (2018) showed that it was possible to implement TT in PIC; however, it depended on staff

engagement and adherence to the predetermined structure of TT. Further, measurements showed questionable effects on the quality of interactions between patients and staff as well as on staff experiences of stress. Patients experienced TT as contributing to hope for recovery (Molin *et al.* 2019). They felt that they were confirmed, that they had participated on equal terms with staff, that they were seen as a human among other humans, that TT offered a break, and that they felt strengthened.

STUDY AIM

The aim of this study was to illuminate staff experiences of introducing and participating in TT.

METHODS

In this qualitative study, data were collected via individual semi-structured interviews with staff (Kvale & Brinkmann, 2009) and analysed using qualitative content analysis (Graneheim *et al.* 2017; Graneheim & Lundman 2004).

Context

Three PIC units in northern Sweden participated in the project. Two units (1 and 2) cared for patients with mental ill health related to substance abuse, and one unit (3) cared for patients with various kinds of acute mental ill health. All units cared for adult patients who were admitted voluntarily or involuntarily. The units had locked doors, and their days were guided by routines and rules. The mean length of stay was 5 days at

- Regular sessions for TT are established in the units on Mondays through Fridays, for a minimum of five hours/week in total.
- During TT, patients and staff engage in joint activities preferably chosen by the patients.
- One or two members of staff is assigned to deal with administrative tasks during the sessions to allow participating staff to join TT without interruption.
- The units are closed to visitors and outpatient care professionals during TT.

FIG. 1 The predetermined structure of Time Together.

units 1 and 2 and 14 days at unit 3. Medical treatment governed the content of care, and organized nursing interventions were limited due to overcrowded units, low staffing, and heavy workload. The units were staffed by enrolled nurses in mental health (ENs), registered nurses (RNs) including some with a postgraduate diploma in specialist nursing in mental health care and a degree of master in nursing (MHN), a unit manager, and physicians. One psychiatrist at each unit was responsible for medical treatment. One unit employed an occupational therapist, and at all units, psychologists, social workers, and physiotherapists could be consulted if needed.

The introduction of Time Together

First, staff at the units were introduced to the background, rationale, and predetermined structure of TT. Two staff members from each unit planned the intervention together with the research team to optimize the realization of TT. At all units, the intervention took the predetermined structure as a foundation but was further tailored to fit the specific unit. This meant that each unit independently decided when TT would be scheduled during the day and had its own range of activities. The activities were discussed and planned in advance, together with patients who were admitted to each unit.

Participants

Staff participating in the overall evaluation of the intervention (cf. Molin *et al.* 2017) were invited to participate in semi-structured interviews. Five staff members from unit 1, six from unit 2, and six from unit 3 agreed, resulting in 17 interviewees; 11 women and six men. Ages ranged from 25 to 65 years (median 39 years). Eight were ENs, eight RNs, and one was an occupational therapist. Two of the RNs were MHN, and two worked as unit managers. The participants work experience ranged from 1 to 40 years. Fifteen of them worked full time, and 16 were permanent staff. In this paper, the group of participants as a whole will be referred to as staff. In case ENs and RNs had different experiences, this will be reported separately. Further, in the findings, all RNs regardless of educational degree will be referred to as RNs to guarantee confidentiality.

Data collection

An assigned staff member, not part of the research team, asked staff members whether they wanted to

participate, and those who agreed were given a scheduled time for the interview. The first author, an experienced MHN with no earlier work or close relationship to the participants, conducted the interviews in a private room at each unit. Staff were asked to talk about what TT meant to them and further how they had experienced introducing and participating in TT. Additional questions encouraged them to expand on their stories. The interviews lasted between 16 and 55 min (median 32 min). All were audio-recorded and transcribed verbatim.

Analysis

Qualitative content analysis was used for analysis (Graneheim *et al.* 2017; Graneheim & Lundman 2004). First, the authors read through all interviews to get a sense of the whole. JM then divided the text into meaning units and condensed them. The condensed meaning units were coded, and codes with similar content were sorted, interpreted, and abstracted into subthemes. For example, codes describing experiences such as *a positive feeling*, *a calmer unit*, and *less interference* formed the subtheme *creating a peaceful atmosphere*. Subthemes with comparable content were further interpreted and abstracted into themes, which were then interpreted into one main theme. During the analysis, the authors made comparisons with the original texts and discussed the analysis. This led to certain adjustments of the subthemes and themes before the research team agreed on the final results.

Ethics

The study was approved by the heads of the relevant clinical departments of psychiatry and the regional Ethical Review Board (ref: 2016/339-31) and was registered at ClinicalTrials.gov (study ID: NCT02981563). Participants were given written and verbal information about the aim of the study and informed that they could withdraw without having to specify the reason and that confidentiality was guaranteed. Informed consent forms were signed by all participants.

RESULTS

TT as an arena for mental health nursing

The analysis revealed that staff experienced TT as an arena for mental health nursing. This main theme is based on three themes, with the first and second

TABLE 1 Overview of the results

Subtheme	Theme	Main theme
Making a joint decision	Laying a framework for success	TT as an arena for mental health nursing
Providing structure and taking responsibility		
Being flexible		
Struggling with time	Feeling burdened	
Handling disagreements		
Bringing us closer together	Fostering relationships	
Creating a peaceful atmosphere		
Discovering fields of application		

themes relating to the introduction of TT and the third relating to participation in TT. An overview of the results is given in Table 1.

Laying a framework for success

Introducing TT required laying a framework for success. To follow the predetermined structure of TT, they needed to make a joint decision, to provide structure and take responsibility, and to be flexible.

Making a joint decision

The staff explained that introducing TT required them as a team to make a joint decision. They felt bad that an intervention was needed to make them socialize with the patients, but they also saw it as good because it made them try together. They hoped that TT would not die away, and as a team, they strived for TT to be part of their offer to the patients and part of their own daily thinking. They experienced a restructuring of their own thoughts; TT led the team to reflect on their work procedures and quickly became a habit with no need to force each other to participate. The staff described how TT facilitated doing things together, with both each other and the patients, and they stated that everyone in their team agreed that TT was part of their work. It had become easier to plan activities for patients, even on other occasions. No other routines were abolished because of TT, and they were surprised to find that they could not remember what tasks had been scheduled at the same time as TT previously. One said *'In some remarkable way we've succeeded! We don't have more time but we've restructured it'*. The staff said it was easy to finish other tasks before performing TT and that they as a team solved situations that could be a barrier to TT.

Providing structure and taking responsibility

Staff described TT as a boost. The structure of TT helped them avoid the risk that activities would be postponed. However, they stated that it would be vulnerable if all responsibility for TT was on the managers and that key persons with special responsibility for TT were needed. These key persons would need to ensure staff adherence with TT by motivating less engaged staff and being in charge of TT. The key persons themselves experienced it as challenging to push sceptical colleagues into TT, but also said that by taking more space, planning activities, and trying to engage others they had experienced personal growth. One said *'It has been positive to realize that I can dare to take more space. I trust myself more; I have managed to take on a bigger role'*.

The staff experienced that TT made them reflect on their daily planning. They expressed a need to plan for TT and said that it worked satisfactorily when they made such plans. Further, it was crucial to inform the patients about delays or cancellations and to ensure that participating staff could stay for the whole session and that no meetings were booked during this time.

Being flexible

The staff said that activities during TT needed to be tailored to the patients, so everyone could choose something. There had to be thought behind every activity, and the staff described how they 'read' the patients to find mutual interests. Activities were also tailored to themselves, and their different interests led to a varied offer for the patients. It was essential that the activities were recognizable and that materials were prepared before the session began. Activities involving scents, such as baking, cooking, brewing coffee, and making popcorn, were appreciated by the patients; card/board games and bingo were good for socializing; and outdoor walks were meaningful for those who preferred exercise. Further, the number of staff needed to be adapted to the number of patients. More staff meant varied activities, and overcrowded units were limited by small multifunctional common areas. However, the staff said that they managed despite these challenges.

The staff expressed a need to actively attract the patients to TT, to take the initiative, and to be flexible. TT was challenging if patients were severely ill or declined to participate. However, some patients who initially declined activities later joined in. Sceptical patients needed to be prioritized, even though it was easier to involve those who were already positive. The

way in which staff approached the patients could determine whether a patient would join in or not. Further, patients would often join in if they heard laughter, which the staff found satisfying.

Feeling burdened

Introducing TT made the staff feel burdened, as they struggled with time and needed to handle disagreements within their own group.

Struggling with time

As staff initially had a hard time seeing how TT would be possible, they questioned how it would be possible to introduce TT at the units. They said that change often brought about an initial deterioration and explained that it was challenging to get the already hectic workdays together. Medical and administrative tasks had priority, and beyond these, there was only time for emergencies. The staff felt unsuccessful, as being together with patients was forgotten. One said *'The administrative work becomes the important work; the invisible work (being with the patients) does not count'*. TT became one more thing on their agenda. However, some found that there was a suitable schedule for TT, and they succeeded without rescheduling.

When staffing was insufficient, finding time for TT was a delicate matter. According to the RNs, the ENs had time for TT but their responsibility for safeguarding the unit prevented them from participating. The RNs felt this was a shame. Further, they said that they arrived late for TT because of their workload and that the added workload of TT meant overtime for them. They experienced TT as a stressful burden that annoyed them. The RNs also described it as unfamiliar to leave their tasks and join in activities with the patients. It was challenging to find peace, but that over time they found it easier to relax.

Handling disagreement

The staff said it was exciting to introduce TT, but they experienced disagreements among themselves. Some expressed critical attitudes against TT. They complained that the patients became *'like a gang'* because of TT and that this constituted a risk; they also said that TT made the units *'too cosy'*, which could lead to patients never wanting discharge. Some staff questioned how much fun the patients should be having and stated that PIC units were not youth centres.

Some RNs said that they did not join in TT because they took pride in their profession and thus worked

with medical/administrative tasks. They were unable to participate because of these tasks, but that if someone else performed them, then they would be able to join in. One said *'I don't miss the ENs when they're doing TT, but they complain that they miss me'*. Some staff experienced restlessness during TT, and some felt that it did not make sense. They described it as challenging to be present during TT as it did not feel like working. They questioned the value of colouring during work hours and likened it to kindergarten. They also questioned why all staff should participate in TT when not all patients did. Other staff experienced resignation when colleagues refused to join. They stated that TT relied on the attitudes of staff and that the managers had responsibility to support the maintenance of TT and counteract resistance.

Another obstacle for TT was having different psychiatrists who worked irregularly and did not commit to unit routines; for example, they might schedule patient meetings during TT. The staff experienced that this disturbed the atmosphere created by TT. On the other hand, TT went smoothly when the psychiatrists worked regularly and understood the intentions of TT. Staff wished for those psychiatrists to join in TT.

Fostering relationships

The staff said that participating in TT fostered their relationships with the patients. TT brought them together and created a peaceful atmosphere at the unit. Further, during TT, the staff discovered fields of applications which they found helpful in caring for the patients.

Bringing us closer together

The staff saw TT as a way to work professionally with the patients and accomplish their key mission: to be there for the patients and to have close contact with them. One said *'That's why we're here; we're here for them'*. They experienced that after introducing TT, there was less friction and the patients had more understanding for them. The staff thought the patients experienced them as fair, which made it easier for them to convince patients to explore activities and strategies for their mental ill health, as well as to set limits for the patients if needed.

The introduction of TT contributed to changing roles between staff and patients. The staff explained that they left their specific roles – an ill patient and a nurse – and gave more of themselves as persons. Some described how they *'reassessed'* the patients as humans

who were more than just a social security number. This was experienced as *'being just like two humans'*, and this perspective contributed to having more control and talking to a person about ordinary things instead of talking to a patient about illness. The staff shared and became more personal with the patients, as if they had become friends, though they emphasized that this was still on a professional basis. They thought this contributed to patients getting a new picture of them and realizing that the staff were fellow humans with a personal side. The staff felt that being two humans on the same level meant a lot to the patients and that well-being by that, took a greater part in the patients care.

The staff also said that TT highlighted the need for sustainable relationships with patients. Joint activities improved the relationships, which contributed to trust. One said *'The invisible work; that is, being with the patients, makes a difference'*. The staff experienced that relationships built during TT had a basis of humanity and sharing, which improved their professionalism. One said *'TT is something that we own together'*.

Creating a peaceful atmosphere

TT brought about a change in the atmosphere on the units, with fellowship and safety developing as more staff socialized in common areas. One said *'Now the patients see that we exist and feel that we are there'*. Being able to laugh got easier, and the atmosphere became softer. TT contributed to a positive spirit that spread like ripples on water, even to those not participating in TT. The staff noticed that the patients were more satisfied and rarely requested PRN medication. Instead they initiated social activities outside TT. TT provided a good end to the shift for those going off-duty, and a calm start for those working the evening shift. The staff concluded that TT contributed to something more than the units had offered before, that the care had improved, and that TT was *'a big thing'* that contributed to *'something really good'*.

The staff described how the introduction of TT relieved their consciences. The RNs said it felt good to know that someone was offering the patients joint activities, even though they had a hectic shift. Staff who joined in TT said that it felt good to do something for the patients, and knowing that the patients were engaged in meaningful activities made it easier to proceed with the next task. Participating in TT also meant relaxation for staff, as TT became a free zone where they could sit in peace. One said *'It's like psychotherapy for yourself; you learn how to relax'*. The staff described being less tired, thriving, feeling more

satisfied and proud with their own efforts, and of what the units had to offer to the patients.

Discovering fields of application

Staff described TT as a nursing activity just as important as serving food. TT helped them in their work, as it allowed them to observe the patient as a whole; it thus became a tool for assessment and a goal in nursing. For example, they could observe how the patients felt and acted in both health and ill health, and hence get a wider picture of them. One said *'You observe the patients recovering. You get to see what makes them shine as they show their healthier aspects during TT'*. The staff were able to see how patients moved from negativity to being proud of themselves, which could be used in their work with the patients. They could challenge the patients during TT, and they found that the patients made greater efforts during this time than at other times during the day. The staff explained that TT created curiosity among the patients, strengthened them, and made them discover new strategies for relieving anxiety. This both fostered the patients' recovery and felt like a victory for the staff.

Conversations took a different form during TT and proceeded with no pressure. Doing things together seemed to help conversations to become natural, and patients opened up more. The content in these conversations was useful later on in the care. One said *'Having conversations and doing activities at the same time is like hitting two targets in one shot'*. The staff stated that colleagues who chose not to participate in TT missed out on such experiences and that they had noticed patients avoiding having conversations with those staff members.

The staff saw that the patients benefited from doing things together, and observing patients interacting with each other contributed to their assessments. These interactions were helpful for those who needed to develop social skills and that this contributed to patients *'feeling normal'* instead of ill, strange, or abnormal. Moreover, problems could be solved when the patients joined in TT; for example, patients could receive answers promptly, and anxious patients relaxed. Patients under constant observation benefited from joining TT, and this also gave staff some relief and facilitated their work. One noted *'TT is like anxiety management for a whole group'*.

It could be difficult to encourage patients to join TT, because of their ill health; some patients became stressed if TT was crowded, and some experienced TT as a constraint or being excluded. The staff stated that

it was their responsibility to counteract this. However, patients preferably looked forward to TT. The staff considered TT to be a boost for the patients, as it enriched their being on the units. One said '*I think that patients should have fun. It can be good for your health, knowing that you can have fun!*' The staff believed that activities were healthy for the patients and that doing things together was part of the patients' recovery. For example, during TT, the patients were distracted from their ill health for a moment, more patients were active, and the arrangement made it easier to get the patients out of their beds. The staff also thought that the patients no longer felt alone, but instead seen and involved.

DISCUSSION

This study revealed that staff in PIC experienced TT as an arena for mental health nursing. The staff prepared for the introduction of TT by laying a framework for success, but the actual implementation meant feeling burdened. However, when evaluating their participation in TT, the staff felt that TT fostered their relationships with the patients.

A successful introduction of TT meant making a joint decision, providing structure, taking responsibility, and being flexible. Key to this was the presence of specific staff members to advocate the intervention. Similar results were found in an evaluation of PET by Dodd *et al.* (2018), which showed a need for someone to make sure that actions were taken in line with the intervention. As described by Molin (2018), MHN are in a central position for taking on such responsibility in the pursuit of this goal. In PIC, MHN have specific responsibility as leaders in nursing practice as well as their own and overall responsibility for nursing. They have advanced knowledge and skills in both nursing theory and interventions, making them obvious leaders for TT to support patients in their personal recovery.

For the staff in our study, TT could mean feeling burdened due to struggles with time and disagreements. Our results show that staff base their work on different assumptions about how care should be conducted and what patients in PIC might need for recovery. An evaluation of PET by Thomson and Hamilton (2012) showed similar results. According to their study, some medical staff disagreed that therapeutic time was essential in supporting patients to recover from mental ill health, and even felt that therapeutic time was a barrier to medical assessments. Our interpretation of the present results is that different assumptions

regarding care create contradictions and thereby become barriers for TT. For example, on the one hand, some questioned how much fun the patients should have during their admission, stating that the units were not youth centres. Conversely, others said that surely the patients should have fun during their admission, as this allowed staff to observe the patients' healthy aspects and what made them shine. There are different assumptions regarding caring approaches and nursing in PIC. Björkdahl *et al.* (2010) used metaphors as 'the bulldozer' and 'the ballet dancer' describing different caring approaches. The former approach related to *guarding the ward* focusing on safety and structure while the latter signalled *concern for the patients as persons* focusing on trust and the desire to provide person-centred care. Salberg *et al.* (2019) described nursing staff's understanding of the real-world phenomenon 'nursing' in the context of PIC and showed different perceptions of nursing described as either '*nursing is to respond to symptoms and behaviours*' and '*nursing is to support vulnerable human beings*' (Salberg *et al.* 2019, p. 1829–1830). Maybe the different assumptions regarding care and nursing represent parts of MHNs responsibility in psychiatric inpatient care. However, Rio *et al.* (2020) argues that a primary risk management focus may prevent MHNs from focusing on the relational aspects of care. We are convinced that focus on relational and person-centred aspects could respond to caring concerns of the patients and thus ward safety. One way for such response could be implementation of joint activities.

Barker and Buchanan-Barker (2005) state that content in care should involve a balance between active engagement in activities and passive periods for natural recovery and healing. However, recent nursing research in the context of PIC reveals imbalances in this matter. For example, Lindgren *et al.* (2015), Molin *et al.* (2016), and Folke *et al.* (2017) found that patients expressed feelings of boredom and 'doing nothing' while admitted to PIC units. Foye *et al.* (2020) state that boredom is a concern in PIC, linked to increased incidents of self-harm and aggression among patients, and that a reduction of boredom could reduce adverse incidence. The opposite of boredom is fun, and over the years, theorists have presented various arguments for patients having fun and play. Freud (1959) and Winnicott (2012) perceived play as a way to master experiences and solve conflicts. Eriksson (1987) argued that playing is an expression of health, joy, and zest for life but also gives space for rest and recovery. Patients' also confirmed that play contributes to health when

participating in TT. Molin *et al.* (2019) showed that participating in TT contributed to hope for recovery, that patients saw TT as offering a break from their mental ill health, and that patients were strengthened by both the activities and the fellowship that developed during TT. Further, patients described that quality time with staff contributed to understanding for the staff members' situation and strengthened the relationship with staff. It is the role of staff to join the patients through the playing process; not only to activate it, but most of all to understand and accept it as part of the health process (Eriksson 1987). Similarly, Barker and Buchanan-Barker (2005) used the concept of bridging to argue that PIC staff are commissioned to reach the person in need through human processes. The initiative for these processes is owned by staff, and in these situations, staff need to make use of their own creativity. For this to be possible, staff need to cope with their own needs and feelings (Barker & Buchanan-Barker 2005). According to Eriksson (1987), as patients have control during play, staff need the ability to bow, to dare, to go beyond themselves, and to use small actions to show that they really care. The present study indicates that for some PIC staff, this might be challenging. This issue is crucial for a successful implementation of TT, as our overall results on TT show that the extent to which PIC will involve playing by the use of TT is determined by engagement from staff.

Conversations are a key tool for staff in PIC, and our results show that conversations during TT took a changed form. Doing things together contributed to natural and relaxed conversations where patients seemed to open up more on difficult topics. Further, the content of these conversations was useful later on in the care. A previous investigation of patients' perspectives on this (Molin *et al.* (2019)) showed that conversations during TT contributed to developing trust in the staff and made the patients feel just like anyone else rather than like a patient undergoing treatment or someone who was abandoned. Those results also showed that the content in conversations during TT shifted from ill health to everyday life topics, leading the patients to experience relief from shame, guilt, and feeling stupid. According to staff in this study, conversations during TT enabled for them to see the patients' healthy side and what made them shine. Consequently, ordinary conversations during TT seem to have relationship-building and health-promoting effects. Our clinical experience is that ordinary conversations are questioned in the context of modern psychiatric care, as they do not fall within the scope of psychotherapy and are not well-evaluated.

However, Barker and Buchanan-Barker (2005) stated that conversations are needed to access a person's story and experiences, and Eriksson (1987) argued that conversation or small talk is needed to both create and maintain caring relationships. Access to a person's experiences and caring relationships are both essential in mental health nursing. Barker and Buchanan-Barker (2005) also argued that a person's personal story is developed through collaboration between the patient and the professional. The story is not a separate monologue, but rather something composed through a series of conversations. Barker and Buchanan-Barker (2005) distinguished these conversations from diagnostic interviewing and psychotherapeutic practice, as in the latter situations, the professional holds the power, guides, and directs the patient. Conversely, during conversation, the power is shared and rests on reciprocity and equality; qualities in line both with mental health nursing and with fostering patients' personal recovery.

METHODOLOGICAL DISCUSSION

This study adds to a broader picture of TT's influence on interactions between patients and staff in PIC, complementing the quantitative results reported by Molin *et al.* (2018) and the qualitative results reported by Molin *et al.* (2019). This demonstrates the value of using a variation of designs when evaluating interventions. As there is no one single truth, measuring effects and exploring experiences instead enables showing a broader picture of the whole. Another aspect of showing a broader picture of TT is the participants' varied experiences of TT. All participants in this study had experience of working on the units before and after the introduction of TT, which made them suitable to evaluate TT. Participants elaborated on their answers in a both condensed and narrative way as shown in the varied length of the interviews. This resulted in wide-ranging experiences of introducing and participating in TT, which represented different PIC settings, leading to variation in data, adding to both credibility and transferability of the results.

The data in this study were co-created between the participants and the interviewer. An interview is a verbal interaction where the interviewer constructs the main questions and the participants, by their answers, influence the follow-up questions (cf. Mishler, 1986). The analysis also comprised a co-creation between the text and the researchers (cf. Graneheim *et al.* 2017). In terms of dependability, the composition of the research team was one strength of this study. Three researchers

had experiences of working in PIC, while one lacked psychiatric experiences on the whole. This broadened the discussions during the analysis. Having varied understandings of a context is an advantage, since 'a text never implies one single meaning, just the most probable meaning from a certain perspective' (Krippendorff, 2013). Thus, our interpretation of how these PIC staff experienced their introduction of and participation in TT is just one possible way to pass on staff members' voices on this matter.

CONCLUSION

TT is a valuable nursing intervention in PIC. According to staff, and despite their different assumptions regarding caring approaches and nursing in PIC, the intervention requires preparation and involves challenges during implementation, but most of all means opportunities to foster relationships between patients and staff. In this way, TT constitutes an arena for mental health nursing where play and conversations based on reciprocity and equality can contribute to patients' recovery.

RELEVANCE FOR CLINICAL PRACTICE

TT is an attempt to cultivate quality interactions between patients and staff in PIC. It has its roots in theories of engagement and reciprocity, but also relies on the belief that what might be perceived as simple – or, as stated in this study, invisible – could be extraordinary. The participants' experiences of TT were in line with our intentions. However, struggling with time and handling disagreements with different assumptions regarding caring approaches and nursing were challenges that counteracted the implementation. To be successful, staff and managers must overcome these challenges. Continuous reflections on nursing and attitudes and adherence to the predetermined structure of TT in both good and challenging times are crucial for maintenance of the intervention.

ETHICAL APPROVAL

The study was approved by the regional Ethical Review Board (ref: 2016/339-31).

REFERENCES

- Barker, P. J. & Buchanan-Barker, P. (2005). *The Tidal Model: A Guide for Mental Health Professionals*. New York: Routledge.
- Björkdahl, A., Palmstierna, T. & Hansebo, G. (2010). The bulldozer and the ballet dancer: aspects of nurses' caring approaches in acute psychiatric intensive care. *Journal of Psychiatric and Mental Health Nursing*, 17, 510–518.
- Cleary, M. (2004). The realities of mental health nursing in acute inpatient environments. *International Journal of Mental Health Nursing*, 13, 53–60.
- Cleary, M., Hunt, G. E., Horsfall, J. & Deacon, M. (2012). Nurse-patient interaction in acute adult inpatient mental health units: A review and synthesis of qualitative studies. *Issues in Mental Health Nursing*, 33, 66–79.
- Dodd, E., Cheston, R., Procter, C. *et al.* (2018). Protected engagement time on older adult mental health wards: A thematic analysis of the views of patients, carers, and staff. *International Journal of Mental Health Nursing*, 27, 608–618.
- Eriksson, K. (1987). *Vårdandets idé (The idea of caring)*. Stockholm: Almqvist & Wiksell.
- Folke, F., Hursti, T., Kanter, J. W. *et al.* (2017). Exploring the relationship between activities and emotional experience using a diary in a mental health inpatient setting. *International Journal of Mental Health Nursing*, 27, 276–286.
- Foye, U., Li, Y., Birken, M., Parle, K. & Simpson, A. (2020). Activities on acute mental health inpatient wards: A narrative synthesis of the service users' perspective. *Journal of Psychiatric and Mental Health Nursing*. <https://doi.org/10.1111/jpm.12595>
- Freud, S. (1959). Creative writers and day-dreaming. In: J. Strachey, A. Freud, A. Strachey & A. Tyson (Eds). *The standard edition of the complete psychological works of Sigmund Freud, volume IX (1906-1908): Jensen's 'Gradiva' and other works* (pp. 141–154). London: The Hogarth Press.
- Gabrielsson, S., Sävenstedt, S. & Olsson, M. (2016). Taking personal responsibility: Nurses' and assistant nurses' experiences of good nursing practice in psychiatric inpatient care. *International Journal of Mental Health Nursing*, 25, 434–443.
- Graneheim, U. H. & Lundman, B. (2004). Qualitative content analysis in nursing research: Concepts, procedures and measures to achieve trustworthiness. *Nurse Education Today*, 24, 105–112.
- Graneheim, U. H., Slotte, A., Säfsten, H. M. & Lindgren, B. M. (2014). Contradictions between ideals and reality: Swedish registered nurses' experiences of dialogues with inpatients in psychiatric care. *Issues in Mental Health Nursing*, 35, 395–402.
- Graneheim, U. H., Lindgren, B. M. & Lundman, B. (2017). Methodological challenges in qualitative content analysis: A discussion paper. *Nurse Education Today*, 56, 29–34.
- Gunasekara, I., Pentland, T., Rodgers, T. & Patterson, S. (2014). What makes an excellent mental health nurse? A pragmatic inquiry initiated and conducted by people with lived experience of service use. *International Journal of Mental Health Nursing*, 23, 101–109.
- Krippendorff, K. (2013). *Content Analysis: An Introduction to Its Methodology*. Thousand Oaks, London: Sage.

- Kvale, S. & Brinkmann, S. (2009). *InterViews: Learning the Crafts of Qualitative Research Interviewing*. Los Angeles: Sage.
- Lindgren, B. M., Aminoff, C. & Graneheim, U. H. (2015). Features of everyday life in psychiatric inpatient care for self-harming: An observational study of six women. *Issues in Mental Health Nursing*, 36, 82–88.
- McAndrew, S., Chambers, M., Nolan, F., Thomas, B. & Watts, P. (2014). Measuring the evidence: Reviewing the literature of the measurement of therapeutic engagement in acute mental health inpatient wards. *International Journal of Mental Health Nursing*, 23, 212–220.
- Mishler, E. G. (1986). *Research Interviewing: Context and Narrative*. Cambridge, MA: Harvard University Press.
- Molin, J. (2018). *Time Together : A nursing intervention targeting everyday life in psychiatric inpatient care : patient and staff perspectives*. Diss. Umeå universitet. <http://urn:nbn:se:umu:diva-147694>
- Molin, J., Graneheim, U. H. & Lindgren, B.-M. (2016a). Quality of interactions influences everyday life in psychiatric inpatient care-patients' perspectives. *International Journal of Qualitative Studies on Health and Well-Being*, 11, 29897.
- Molin, J., Graneheim, U. H., Ringnér, A. & Lindgren, B.-M. (2016b). From ideals to resignation – interprofessional teams perspectives on everyday life processes in psychiatric inpatient care. *Journal of Psychiatric and Mental Health Nursing*, 23, 595–604.
- Molin, J., Lindgren, B.-M., Graneheim, U. H. & Ringnér, A. (2017). Does 'Time Together' increase quality of interaction and decrease stress? A study protocol of a multisite nursing intervention in psychiatric inpatient care, using a mixed method approach. *British Medical Journal Open*, 7, e015677.
- Molin, J., Lindgren, B.-M., Graneheim, U. H. & Ringnér, A. (2018). Time Together: A nursing intervention in psychiatric inpatient care: Feasibility and effects. *International Journal of Mental Health Nursing*, 27, 1698–1708.
- Molin, J., Graneheim, U. H., Ringnér, A. & Lindgren, B. M. (2019). Patients' experiences of taking part in Time Together–A nursing intervention in psychiatric inpatient care. *International Journal of Mental Health Nursing*, 28, 551–559.
- Nolan, F. M., Fox, C., Cheston, R. *et al.* (2016). A feasibility study comparing UK older adult mental health inpatient wards which use protected engagement time with other wards which do not: Study protocol. *Pilot and Feasibility Studies*, 2, 7.
- Polacek, M. J., Allen, D. E., Damin-Moss, R. S. *et al.* (2015). Engagement as an element of safe inpatient psychiatric environments. *Journal of the American Psychiatric Nurses Association*, 21, 181–190.
- Rio, J. H., Fuller, J., Taylor, K. & Muir-Cochrane, E. (2020). A lack of therapeutic engagement and consumer input in acute inpatient care planning limits fully accountable mental health nursing practice. *International Journal of Mental Health Nursing*, 29, 290–298.
- Salberg, J., Bäckström, J., Röing, M. & Öster, C. (2019). Ways of understanding nursing in psychiatric inpatient care–A phenomenographic study. *Journal of Nursing Management*, 27, 1826–1834.
- Thomson, L. D. A. & Hamilton, R. (2012). Attitudes of mental health staff to protected therapeutic time in adult psychiatric wards. *Journal of Psychiatric and Mental Health Nursing*, 19, 911–915.
- Turesson, H., Eklund, M. & Wann-Hansson, C. (2011). Perceived stress among nursing staff in psychiatric inpatient care: The influence of perceptions of the ward atmosphere and the psychosocial work environment. *Issues in Mental Health Nursing*, 32, 441–448.
- Winnicott, D. (2012). *Playing and Reality*. Abingdon, UK: Taylor and Francis.